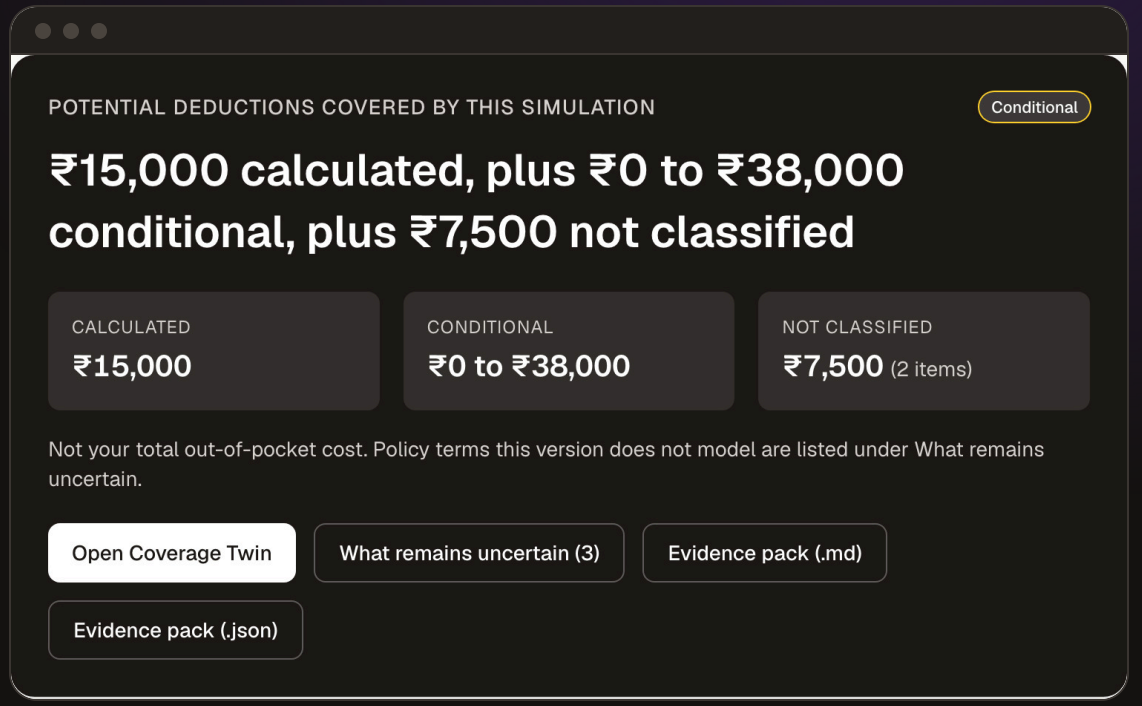
TRACK 2 · AI-POWERED FINANCIAL JOURNEYS

Claim Sach

Coverage Twin: know what your policy might leave you paying, *before* admission.

Pushkar Mondal · Founding Engineer, Xeliport



The first thing a user sees: calculated, conditional and unclassified amounts kept apart, never folded into one total.

CONTENTS

Seven sections, eleven problems, one working product

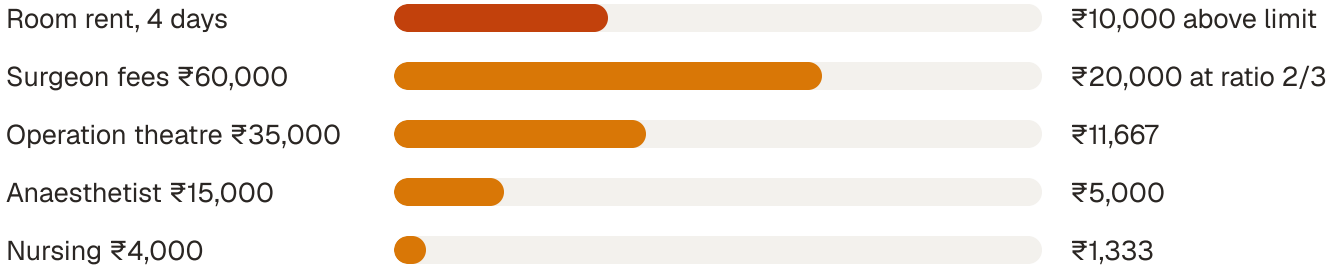
01	**Problem** slides 3 to 4	The policy's cost shows up at the hospital; the eleven things that go wrong, mapped to what the product does
02	**Solution** slides 5 to 6	Documents in, itemised deductions out; change your room, see the difference
03	**Proof** slides 7 to 15	Nine screens of the working product, one problem each, with the clause behind every number
04	**Technology** slides 16 to 17	A deterministic engine, AI only with human review, and the guarantees as they stand in the code
05	**USP** slide 18	Item-level, clause-cited, honest about unknowns
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07	**Business model** slide 20	Where it fits Paytm, and the tests before anyone believes a number

Every entry above is a link, and every slide's footer links back here. Every screenshot is the product as it runs today, uncropped at element level. Source links open the repository and the regulator's copy of the policy wording.

The policy's cost shows up at the hospital

Room-rent limits, proportionate deductions, ICU limits, co-payments, sub-limits and exclusions surface at admission, not at purchase. Pick a room above the eligible rate and many policies pay the surgeon, the anaesthetist, nursing and the operation theatre only in the proportion **eligible ÷ actual rate**. It is not about the room.

One room choice, four items that are not the room



Worked example on a synthetic policy and estimate, computed by the prototype. Not a market statistic.

₹2,500 a day

the room-rent gap: ₹7,500 chosen against ₹5,000 eligible

₹48,000

potential deductions from that one choice; ₹38,000 of it on items that are not the room

4 of 11 lines

affected under the policy's definition; 3 excluded by it; 2 the wording never classifies

Which items are touched depends on the policy's own definition, on facts only the hospital knows, and on clauses the policyholder has never read. General explainers stop at the concept. The gap we are testing is applying *your* wording to *your* bill.

Eleven problems, and what the product does about each

#	WHAT GOES WRONG FOR THE POLICYHOLDER	WHAT CLAIM SACH DOES, AS BUILT	WHERE · SLIDE
01	A room above the eligible rate cuts the surgeon's fee, not just the room	Applies the proportion (eligible ÷ actual) item by item, with the ratio and the clause on each row	Coverage Twin · 6
02	Which items are cut depends on this wording's own definition	Every line classified by the policy's text: included, excluded, or unknown; never one ratio over the whole bill	Twin · 7
03	A fact only the hospital knows: does it bill differently by room?	A range with both bounds until it is confirmed; the answer is recorded as yours in every trace	Uncertain, Input review · 8
04	Items the wording never mentions get quietly treated as ₹0	Unknown is never zero: shown apart from every total, with who can settle it; the not-modelled list is on screen	Uncertain · 9
05	ICU rates and capped percentage limits behave differently from room rent	Lower of percent and cap; ICU can trigger the proportion; two different ratios give bounds, not a guess	Real wording · 10
06	A co-payment stacks on everything else	A bounded range until every other amount is settled; a single figure only then	Real wording · 10
07	Old wordings conflict with later IRDAI norms	A caveat on the affected items: requires insurer confirmation, with the circular named	Real wording · 10
08	“How did you get this number?”	A trace on every amount: clause and page, inputs with provenance, formula, result; tampered traces fail validation	Trace drawer · 11
09	Encoding a new wording takes an expert	Find the clause by keyword; a model proposes rules with quotes, the engine verifies each quote verbatim, a person confirms	Ruleset builder · 12
10	Your real documents, not a demo, and no server to trust	JSON and CSV read in the browser only; schema-checked; misquotes refused; every field correctable with provenance	Input review · 13
11	Which room, what to ask, and how to prove it later	Room ladder, question sheet per hospital and insurer, why-this-changed, and an evidence pack with every trace	Ladder, sheet, pack · 14, 15

Documents in, itemised deductions out, every step on screen

1 **Policy documents**
Wording and schedule, synthetic or real, labelled which.

2 **Structured rules**
Each rule cites its clause and quotes it in full.

3 **Scenario simulation**
Room, rate, days, the hospital's own line items.

4 **Itemised potential deductions**
Room excess, ICU excess, proportion, co-payment.

5 **Source clause**
Page-cited, verbatim, beside every number.

6 **Explicit uncertainty**
What is open, what is at stake, who can settle it.

0:00
Overview
₹15,000 calculated, ₹0 to ₹38,000 conditional, ₹7,500 not classified. Not a total, and it says so.

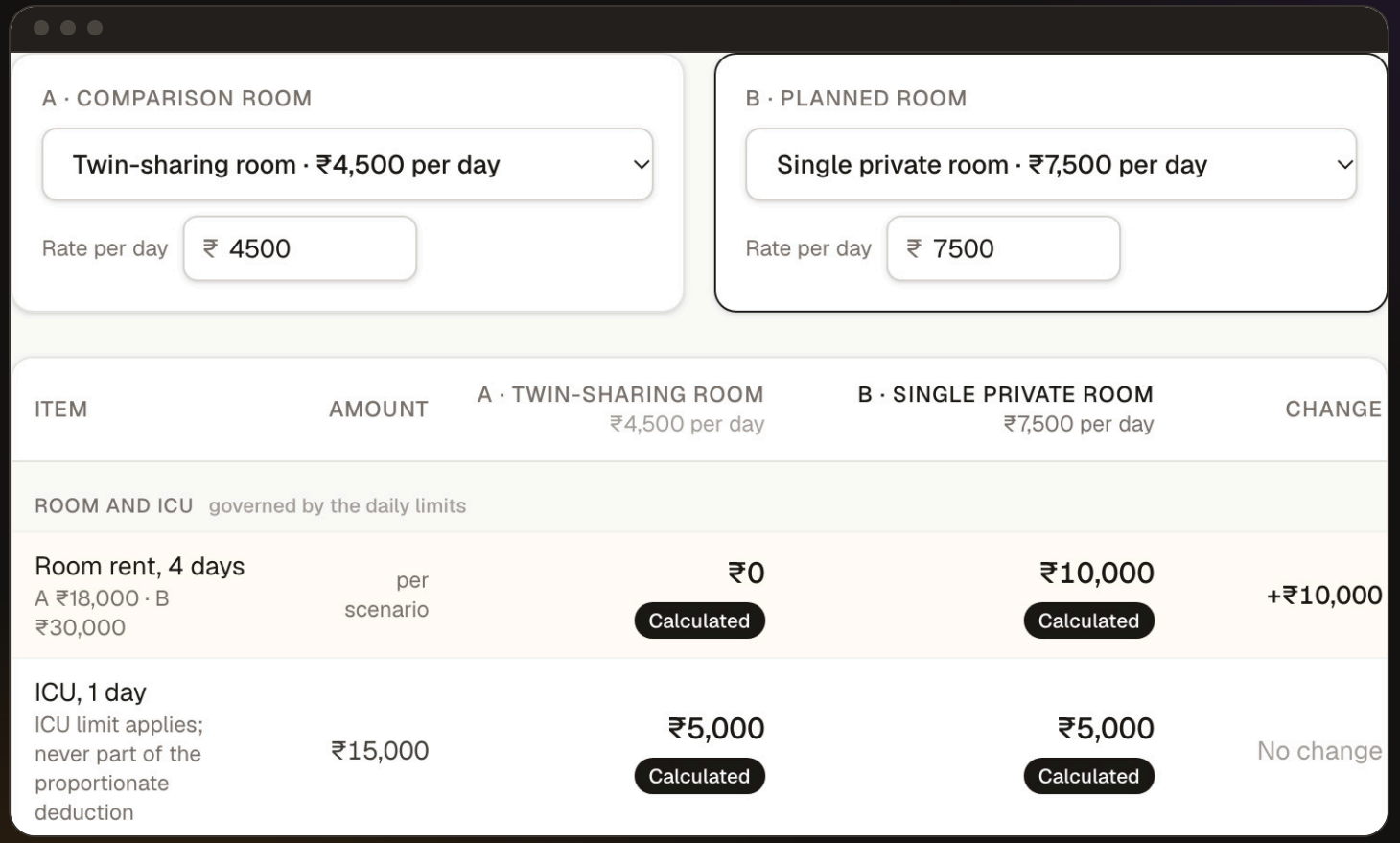
0:25
Twin
Change the room; the itemised breakdown updates at once with the cause rule per row.

1:35
Uncertain
Ask the hospital, ask the insurer, resolve now; plus what this version does not model at all.

3:00
Real wording
Switch to Arogya Sanjeevani and watch the same engine refuse to guess where the wording is silent.

Not a chatbot. Not a payout guarantee. A scenario estimate of the deductions it covers, with its unknowns on screen. Six screens, two wordings, one command, offline.

Change your room. See the difference.



Twin-sharing (A) against single private (B), same policy, same estimate. The room row changed: ₹0 against ₹10,000 above the limit, clause C1. The ICU row did not: its limit never depends on the room.

1. **One policy, one estimate, two rooms**
   Only the rows a rule touched light up. The rest stay exactly as they were.

2. **Every amount opens its evidence**
   Clause and page, inputs with where they came from, the formula, the result.

3. **Ranges, not guesses**
   Until the hospital confirms differential billing, the surgeon's deduction is ₹0 to ₹20,000.

4. **Unknown is never zero**
   Lines the wording never classifies stay out of every total, and the panel says who can settle them.

Which items are cut is the wording's call, not ours

PROBLEM 2 THE POLICY'S OWN DEFINITION DECIDES

NOT SUBJECT TO THE PROPORTIONATE DEDUCTION				
Implant (knee prosthesis)	₹1,20,000	₹0 Not subject to deduction	₹0 Not subject to deduction	No change
Pharmacy and consumables	₹25,000	₹0 Not subject to deduction	₹0 Not subject to deduction	No change
Diagnostics	₹12,000	₹0 Not subject to deduction	₹0 Not subject to deduction	No change
NOT CLASSIFIED BY THE WORDING				
Physiotherapy	₹6,000	— Cannot determine	— Cannot determine	—
Hospital admin charges	₹1,500	— Cannot determine	— Cannot determine	—

The lower half of the same Coverage Twin table: three lines the demo wording keeps whole, two it never classifies. Rows that cannot be determined show a dash, never ₹0.

Included: by the wording's definition

The synthetic clause C4 names nursing, surgeon, anaesthetist, consultant and operation-theatre charges. Arogya Sanjeevani's Note (b) says "all other expenses incurred at the Hospital, with the exception of cost of medicines". Same engine, two different lists.

Excluded: by the same wording

Implant, pharmacy and diagnostics stay whole under C4. Under the real wording only medicines are excepted, so implants and diagnostics are reduced, with a caveat (slide 10). No universal Indian policy is hard-coded anywhere.

Unknown: out of every total

Physiotherapy and hospital admin charges are neither named nor excluded by C4. They are shown separately with "Cannot determine". On Input review you can classify them yourself; the choice is recorded as your assumption in every trace.

A hospital fact turns a range into a number

Ask the hospital

A fact about billing practice that only the hospital can state.

The estimate does not state whether the hospital bills differently by room category. C3 applies only where it does. Until this is established, the proportionate deduction is between ₹0 and ₹38,000.

Affects Nursing charges, 4 days, Surgeon fees, Anaesthetist fees, Operation theatre charges

At stake ₹0 to ₹38,000 · Not established whether the hospital follows differential billing by room category: low = no deduction (C3 does not apply), high = the full proportionate deduction

Resolves it Ask the hospital's billing desk whether the room category changes the rates of these items, then record the answer on Input review.

`DIFFERENTIAL_BILLING_NOT_ESTABLISHED`

Open question, four items affected, ₹0 to ₹38,000 at stake, and what to ask.

ASSOCIATED MEDICAL EXPENSES as defined by C4

Nursing charges, 4 days	₹4,000	₹0 Calculated	₹1,333 Calculated	+₹1,333
Surgeon fees	₹60,000	₹0 Calculated	₹20,000 Calculated	+₹20,000
Anaesthetist fees	₹15,000	₹0 Calculated	₹5,000 Calculated	+₹5,000
Operation theatre charges	₹35,000	₹0 Calculated	₹11,667 Calculated	+₹11,667

After "Yes, confirmed by me" on Input review: the same four rows, now calculated.

PROBLEM 3 DIFFERENTIAL BILLING BY ROOM CATEGORY

Before: both bounds, each with its own trace

Clause C3 applies only where the hospital bills by room category; the estimate does not say. The surgeon's fee shows ₹0 to ₹20,000: low = C3 does not apply, high = the full proportion. The headline keeps it out of the calculated figure.

After: one answer, recorded as yours

Confirm it on Input review and every dependent amount recomputes: calculated deductions move from ₹15,000 to ₹53,000, the open questions drop from 3 to 2, and every trace shows the input as "confirmed by you".

Ranges only with defensible bounds

A yes-or-no fact has two computable outcomes, so both are shown. An item the wording never classifies has none, so it stays unresolved and out of every total.

Unknown is never zero

You can resolve this now

Confirm or classify on Input review. Your answer is recorded as yours in every trace.

The wording does not say whether Physiotherapy (₹6,000) and Hospital admin charges (₹1,500) count as expenses the deduction touches, and does not exclude them either. These items are shown separately and are not part of any total.

Affects Physiotherapy, Hospital admin charges

Resolves it Classify the item on Input review (recorded as your own assumption), or ask the insurer how it treats this charge.

Resolve on Input review

ITEM_NOT_CLASSIFIED

Two lines the wording never classifies, shown apart with the reason and the fix.

Not modelled by this version

These terms can change what you pay. The simulator does not evaluate them, which is why its result is never your total out-of-pocket cost.

- Co-payment clauses
- Sub-limits other than the room rent and ICU limits (disease- or procedure-specific limits, package rates)
- Exclusions and waiting periods
- Non-payable items and consumables lists
- Pre- and post-hospitalisation expenses
- Network, cashless and package-rate differences
- The Sum Insured cap, beyond a notice when the scenario total exceeds it

Every term this version does not evaluate, where the user cannot miss it.

PROBLEM 4 SILENCE IN THE WORDING IS NOT ₹0

STATE	MEANING, AND HOW IT IS SHOWN
Calculated	Every input established or confirmed by you, all conditions met. Plain ink; click for the trace.
Conditional	The rule applies if a named condition holds. Amber badge; a range with both bounds.
Cannot determine	The documents lack or contradict an input the rule needs. Grey badge; a dash, never 0.
Not subject to deduction	The policy's own definition keeps the item out of this rule. Neutral badge; the item stays in the bill.
Requires insurer confirmation	The policy is silent, or only the insurer can settle it. Blue badge; listed with what to ask.
Review required	The schedule contradicts the wording. No amount until a person looks.

In the code, Money has no unknown value: an unknown amount is the absence of a Money plus an uncertainty code. A total with an unknown inside reads "₹15,000 calculated, plus ₹7,500 not classified", never one figure.

Four more mechanisms, one real wording

PROBLEMS 5 · 6 · 7 ICU AND CAPPED LIMITS · CO-PAYMENT · AN OLD WORDING

ASSOCIATED MEDICAL EXPENSES as defined by 4.1.1 Note (b)				
Nursing charges, 4 days	₹4,000	₹1,333 Calculated	₹1,333 Calculated	No change
Surgeon fees	₹60,000	₹20,000 Calculated	₹20,000 Calculated	No change
Anaesthetist fees	₹15,000	₹5,000 Calculated	₹5,000 Calculated	No change
Operation theatre charges	₹35,000	₹11,667 Calculated	₹11,667 Calculated	No change
Implant (knee prosthesis)	₹1,20,000	₹0 to ₹40,000 Conditional	₹0 to ₹40,000 Conditional	No change
Diagnostics	₹12,000	₹0 to ₹4,000 Conditional	₹0 to ₹4,000 Conditional	No change
Physiotherapy	₹6,000	₹2,000 Calculated	₹2,000 Calculated	No change

Arogya Sanjeevani, New India Assurance, UIN NIAHLIP20165V011920, from the regulator's copy: ten clauses verbatim, page-cited, SHA-256 on screen. Partially encoded; the schedule is synthetic.

Capped percentage limits

Room 2% of the sum insured up to ₹5,000 a day; ICU 5% up to ₹10,000. The eligible rate is the lower of the two; the trace shows both.

The ICU surprise

A ₹15,000 ICU day exceeds its cap, so Note (b) applies the proportion even in the twin-sharing room: the surgeon's ₹20,000 shows in both columns. Different room and ICU ratios give bounds, not a number.

Co-payment as a bounded range

5% of the admissible amount: ₹9,900 to ₹13,425 while implants and diagnostics are open; one figure only when everything else is settled.

A 2019-20 wording against IRDAI's 2020 norms

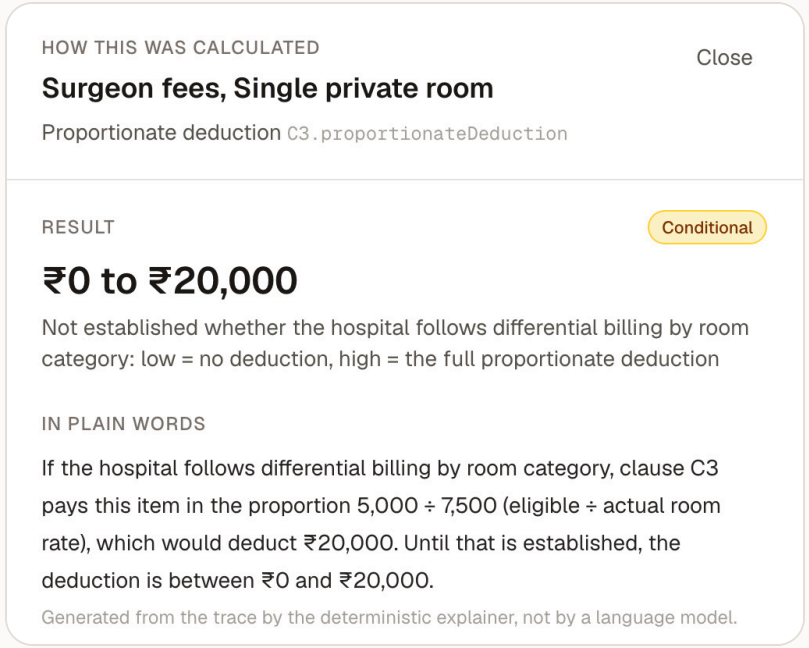
This text reduces implants and diagnostics; IRDAI's circular of 11 June 2020 later barred that. Marked ₹0 to ₹40,000 and ₹0 to ₹4,000, requires insurer confirmation, circular named.

Every number opens the calculation behind it

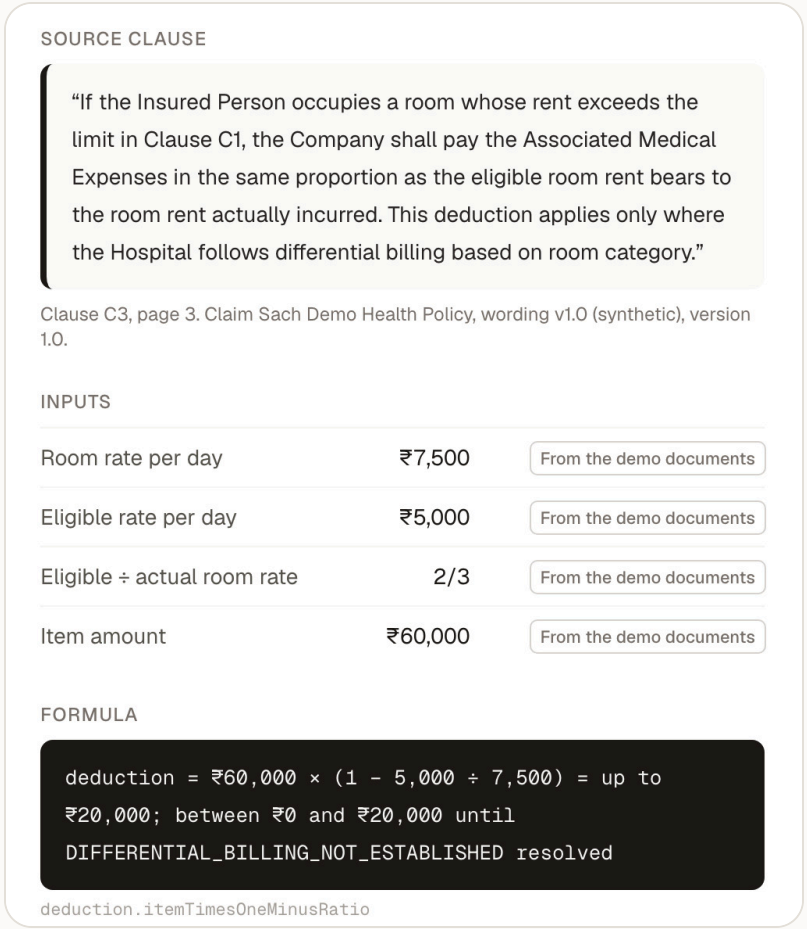
PROBLEM 8 “HOW DID YOU GET THIS?”

- The rule, by id and name.
- Each input with its unit and where it came from: the documents, your upload, or you.
- The source clause, quoted verbatim with document, version and page.
- The formula as text, and by id in the tests.
- The result in paise, shown in rupees, with its state.
- Anything the rule needed that is not established, by code.

The plain-words paragraph is generated from the trace by a deterministic explainer, not by a language model. The evidence pack carries every trace in full.



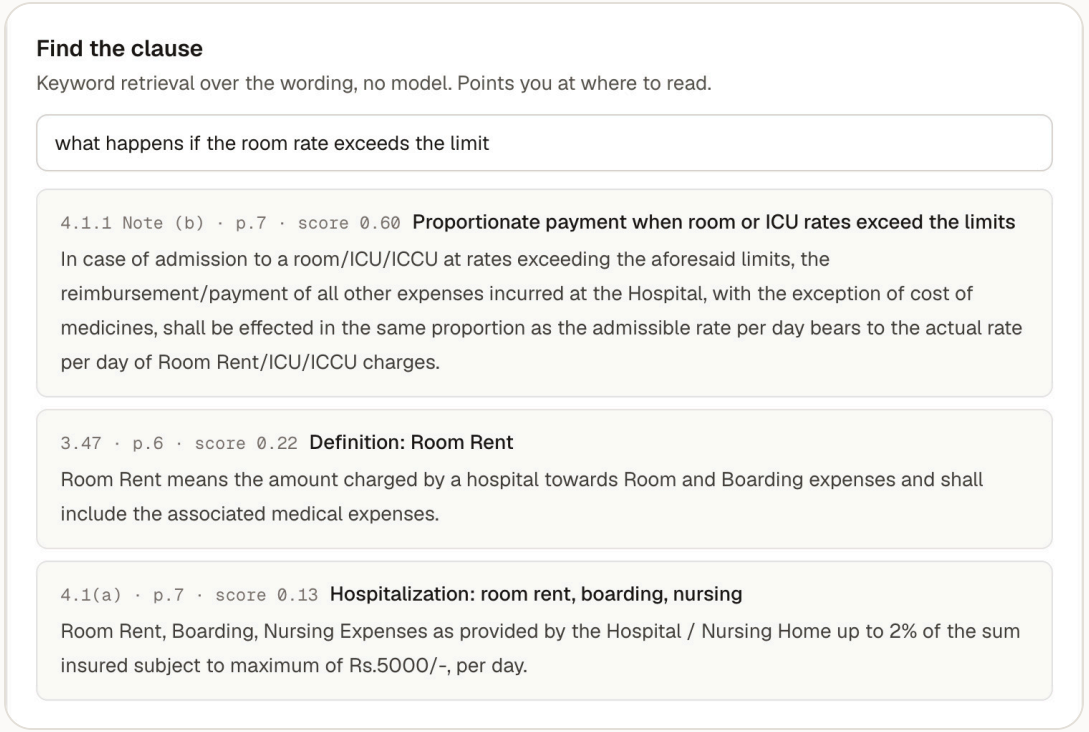
The drawer: result, state, and the explanation in plain words.



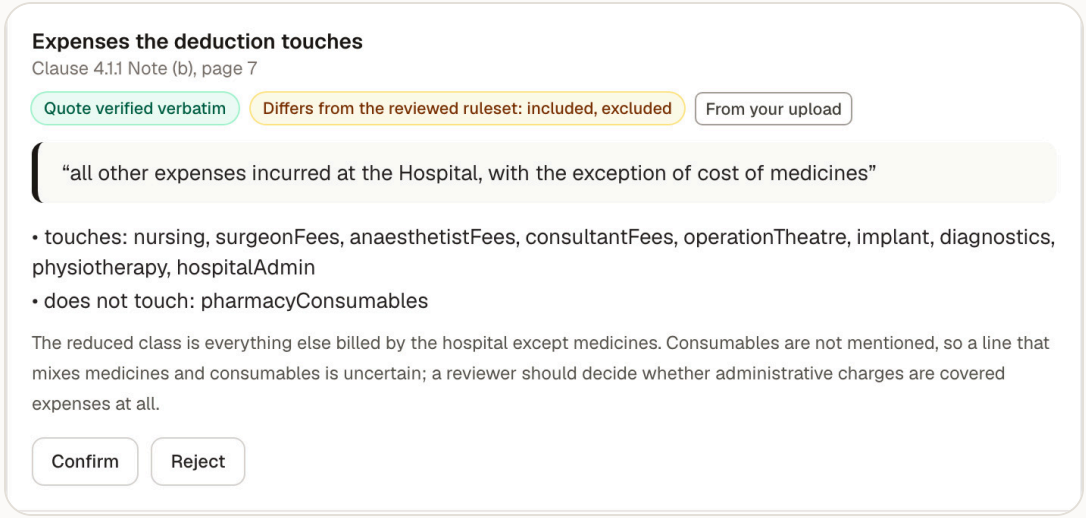
Then the clause, the inputs with provenance, and the formula with the open code.

Model proposes, engine checks, you confirm

PROBLEM 9 ENCODING A NEW WORDING



Find the clause: keyword retrieval over the wording, no model, scored and page-cited. It points a reviewer at where to read.



One cached proposal for Arogya Sanjeevani. Three checks on every card: the quote verified verbatim against the clause; agreement with the human-reviewed ruleset, and here it differs; and its provenance.

The boundary

Confirmed, verified proposals become the active ruleset with the full clause as evidence; a misquote cannot be confirmed. Proposals here are cached and say so; the live adapter returns 503 unless configured and was not exercised. The model never calculates and never sees a schedule, a bill or a name.

Your own documents, and where every value came from

PROBLEM 10 REAL DOCUMENTS, NO SERVER TO TRUST

Policy wording in use

Two wordings ship in the box: the synthetic demo policy, and a real IRDAI-hosted wording encoded partially and quoted verbatim.

- ○ **Claim Sach Demo Health Policy (synthetic)**
  Invented wording with a defined Associated Medical Expenses class and a differential-billing condition. Not issued by any insurer.
- ◉ **Arogya Sanjeevani, The New India Assurance (IRDAI-hosted, partially encoded)**
  A real wording, UIN NIAHLIP20165V011920, downloaded from irdai.gov.in and quoted verbatim. Only the room, ICU, proportionate-payment and co-payment clauses are encoded. The schedule is synthetic.

Verification record

Fetched 2026-10-24 from irdai.gov.in, 31 pages. SHA-256 4e261c78014a210c…. pdftotext (poppler), layout mode; line breaks joined, repeated spaces collapsed.
Encoded clauses: 2, 3.11, 3.27, 3.47, 4.1(a), 4.1(b), 4.1(c), 4.1(d), 4.1.1 Note (b), 9.5. Partial encoding of a real wording hosted by IRDAI. Only the listed clauses are encoded; the rest of the policy is not modelled. The policy schedule used with it in the demo is synthetic.

Input review with the real wording selected: the verification record carries the fetch date, source, page count, SHA-256, extraction method and the list of encoded clauses.

Upload, in the browser only

A ruleset (JSON), a schedule (JSON) and a hospital estimate (JSON or CSV), up to 5 MB each, read with the File API and never sent anywhere. Schema-checked; a rule that misquotes its clause is refused.

Provenance on every field

Every value carries a badge: demo documents, your upload, confirmed by you, or entered by you. Correct one and every trace updates. No account, key, analytics or storage.

Demo case

Switch the seeded documents to see how the engine refuses to overstate when something is missing or contradictory.

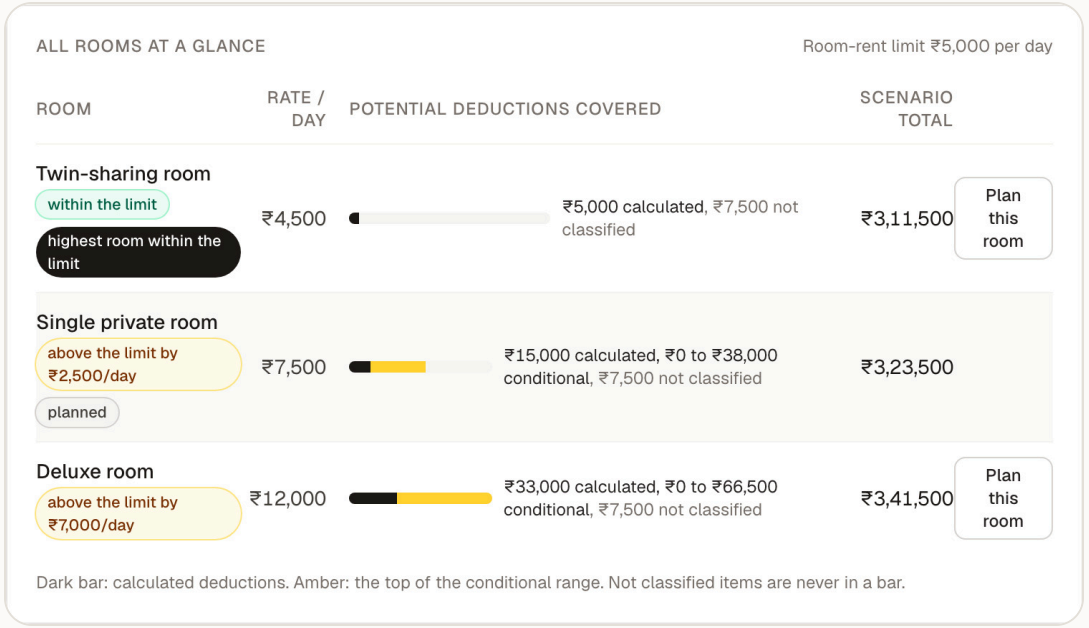
- ◉ **Standard demo case**
  Wording, schedule and estimate as seeded.
- ○ **Policy schedule missing**
  The wording refers to the schedule for the Sum Insured; without it no limit can be determined.
- ○ **Schedule contradicts wording**
  The schedule prints ₹6,000 per day while the wording gives 1% of ₹5,00,000. Review required.
- ○ **Wording without a deduction clause**
  The same policy without clause C3: the room limit still applies, the proportionate deduction does not.

Three failure cases ship as demo variants: the engine refuses to overstate.

Decide the room, then know what to ask

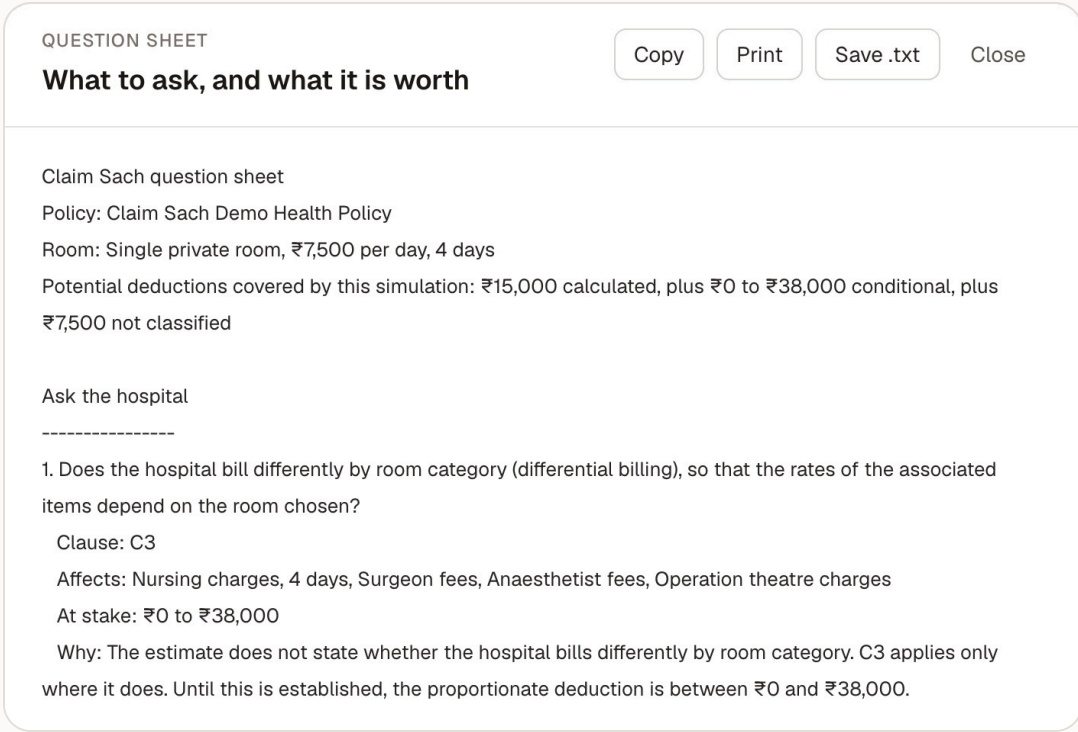
Room ladder

Every room on one view with the deductions at each; the highest room within the limit is marked. One click plans it. The product shows; it does not recommend.



Question sheet

Every open uncertainty becomes a question for the hospital or the insurer, with its clause and the rupees at stake. Copy, print or save; take it to the billing desk.



PROBLEM 11 WHICH ROOM, AND WHAT TO ASK

Why it changed, and the evidence to take with you

ROOM RENT	SCENARIO TOTAL	CALCULATED DEDUCTIONS	CONDITIONAL DEDUCTIONS
+₹12,000	+₹12,000	**+₹10,000**	None → ₹0 to ₹38,000

RULE

Room rent limit: amount above the limit `C1.roomRentLimit`

SOURCE CLAUSE

> "Room rent and boarding charges incurred during Hospitalisation are payable up to 1% of the Sum Insured per day, as stated in the Policy Schedule."

Clause C1, page 2. Claim Sach Demo Health Policy, wording v1.0 (synthetic).

WHAT DIFFERS BETWEEN A AND B

Input	A	B
Room rate per day	₹4,500	₹7,500

`A: excess = max(0, ₹4,500 − ₹5,000) × 4 days = ₹0`

`B: excess = max(0, ₹7,500 − ₹5,000) × 4 days = ₹10,000`

AFFECTED ROWS

Item	A	B	Change
Room rent above the limit, 4 days	₹0 Calculated	₹10,000 Calculated	+₹10,000

Per rule: the clause, the inputs that differ, both formulas, the affected rows.

```
# Claim Sach evidence pack
## Scenario
- Room: Single private room, ₹7,500 per day, 4 days (room rent ₹30,000)
## Potential deductions covered by this simulation
**₹15,000 calculated, plus ₹0 to ₹38,000 conditional, plus ₹7,500 not classified**
(overall state: conditional). Not the total out-of-pocket cost.
## Items
| Item | Amount | Classification | Deduction | State |
| Surgeon fees | ₹60,000 | includedInDeduction | ₹0 to ₹20,000 | Conditional |
| Physiotherapy | ₹6,000 | unknown | — | Cannot determine |
… 8 rows omitted …
## Traces
### Room rent limit (C1.roomRentLimit)
> Room rent and boarding charges incurred during Hospitalisation are payable up to
1% of the Sum Insured per day, as stated in the Policy Schedule.
Formula: `eligiblePerDay = 1% × ₹5,00,000 = ₹5,000 per day`
Result: ₹5,000 (Calculated)
… every other trace follows, then the verification record and the disclaimer …
```

Verbatim lines from the exported Markdown evidence pack (JSON is one click away); "…" marks omitted lines.

Deterministic engine, AI only with human review

Frontend

TypeScript, strict. Next.js 16 (App Router), React 19, Tailwind 4. Six screens on a tested screen model; no business logic in components.

Rules engine

Pure TypeScript, no I/O, no model. Integer paise, exact ratios, one half-up rounding. A trace on every amount: rule, inputs, clause, formula, output, open assumptions.

Two wordings

A synthetic demo policy, and Arogya Sanjeevani (New India, from irdai.gov.in): ten clauses quoted verbatim, page-cited, SHA-256 shown. Partially encoded and labelled so.

AI, in its place

The Ruleset builder: a model proposes rules with quotes; the engine verifies each quote verbatim; a person confirms. Cached in this build; live adapter off. Never in the calculation path.

Quality

Vitest, 180 tests in 18 files, written first. GitHub Actions on every pull request. Evaluation harness: six measures over two wordings, results committed and checked for staleness.

Runs anywhere

One command, offline on a laptop; fonts bundled; Vercel-ready. Nothing the user enters leaves the browser. No key, no account, no analytics, no database.

DATA FLOW, ALL IN THE BROWSER

```
fixture or upload (JSON, CSV) → ingestion: schema + verbatim evidence check → screen model → rules engine: classify → limits → proportion → co-payment → traces → six screens, room ladder, question sheet, evidence pack
```

AI seam, off by default: public clause text → model proposals → verbatim quote check → human confirmation → ruleset. No schedule, bill or name ever leaves the browser.

Six states, eleven named uncertainties, no silent zeros

```ts
export type ResultState =
  | "calculated"
  | "conditional"
  | "unknown"
  | "excluded"
  | "requiresInsurerConfirmation"
  | "reviewRequired";

export type UncertaintyCode =
  | "SCHEDULE_MISSING"
  | "SCHEDULE_CONTRADICTS_WORDING"
  | "DIFFERENTIAL_BILLING_NOT_ESTABLISHED"
  | "ITEM_NOT_CLASSIFIED"
  | "WORDING_SILENT_ON_ROOM_UNAVAILABILITY"
  | "RULE_ABSENT"
  | "LIMIT_RULE_ABSENT"
  | "ICU_RATE_NOT_STATED"
  | "RATIO_AMBIGUOUS_ROOM_AND_ICU"
  | "WORDING_PREDATES_2020_NORMS"
  | "POLICY_CAVEAT";

// src/engine/types.ts, verbatim. Every uncertainty the
// engine can raise is a named code; every amount has a state.
```

Integer paise

Money is a branded integer of paise. mulRatio multiplies with BigInt and rounds half up once per operation; nothing is a float.

Exact ratios

Eligible ÷ actual stays a reduced fraction (gcd), so 5,000 ÷ 7,500 is 2/3, not 0.6667.

No universal policy

Each wording is its own ruleset with its own definitions. No rule is hard-coded; no single ratio is ever applied to a whole bill.

Unknown has no Money

An unknown amount is the absence of a Money plus an uncertainty code. It cannot be added, so it cannot be totalled by accident.

Traces are validated

validateTrace checks each trace against the verbatim clauses and schedules; a tampered or misquoted trace fails. Uploaded rulesets get the same check.

Tests first

180 Vitest tests in 18 files, written before the code. CI runs lint, typecheck, tests, build and the evaluation harness on every pull request, and fails if the committed results are stale.

Item-level, clause-cited, honest about unknowns

1 **Every number is clickable**

Clause and page, inputs with where they came from, the formula, the operation. Tampered traces fail validation.

2 **Unknown is never zero**

Items the wording does not classify stay out of every total. A conditional amount shows both bounds and the condition.

3 **Change your room, see the difference**

Two rooms side by side, the rule behind every changed row; the ladder and the question sheet turn it into a decision and a call script.

4 **Proven on a real wording, honestly**

Arogya Sanjeevani, ten clauses verbatim with a checksum; where the 2019-20 text conflicts with IRDAI's 2020 norms, the engine says so.

What we claim

Claim Sach turns a policy wording into a verifiable hospital-bill simulation: change the room, inspect every modelled deduction, and see what the documents cannot determine.

What we do not claim

Not the first to read clauses; not a payout guarantee; no accuracy figure beyond the measured ones. A bounded review of nine public sources found no tool doing this; we claim we looked, not that nobody does.

The room decision, made before admission

For the policyholder

The room decision with its deductions visible before admission, not after. Questions for the hospital and the insurer written out: what to ask, and what it is worth.

For a broker's claim-assistance agent

The clause on screen during the call. The uncertainty list is the call script; the evidence pack is the written answer.

Room rent	₹18,000	₹30,000	+₹12,000
Scenario total non-room items plus room rent	₹3,11,500	₹3,23,500	+₹12,000
Calculated deductions	**₹5,000**	**₹53,000**	+₹48,000
Conditional deductions	None	None	
Not classified shown separately, never totalled	₹7,500	₹7,500	
POTENTIAL DEDUCTIONS COVERED BY THIS SIMULATION	₹5,000 calculated, plus ₹7,500 not classified	**₹53,000 calculated, plus ₹7,500 not classified**	

One row decides it: ₹5,000 against ₹53,000 of calculated deductions once billing practice is confirmed.

MEASURED, NOT CLAIMED

MEASURE	SYNTHETIC	REAL WORDING
Clause retrieval, top-1	11 of 12	10 of 12
Rule extraction, field-level	10 of 10	10 of 12
Calculations, exact paise	9 of 9 · 46 amounts	3 of 3 · 21 amounts
Uncertainty and refusal	6 of 6	3 of 3
Proposal quotes verbatim (cached)	n/a	6 of 6
Proposals agree with reviewer (cached)	n/a	11 of 13

Measured on the synthetic wording and on ten encoded clauses of one real wording; proposals are cached, not a live run. No accuracy figure appears in the product. Every figure is a potential deduction covered by the simulation, never total out-of-pocket cost.

Where it fits Paytm, and what must be tested

VERIFIED FACTS

- Paytm Insurance Broking Pvt Ltd is a Direct (Life & General) broker: IRDAI Reg. No. 700, renewed 9 Feb 2026, valid to 16 Feb 2029.
- It distributes car, two-wheeler, health, life and term policies from 18+ insurers, including Niva Bupa and Aditya Birla Health. It does not underwrite.
- It already offers claim assistance by phone. Arogya Sanjeevani is IRDAI's standard product on one wording; one verified encoding serves every insurer that sells it.
- FY25 turnover ₹37.1 crore, loss ₹13.2 crore (Form AOC-1), inside a ₹2,593 crore financial-services line in FY26. Insurance is a small line for Paytm today; clarity is a wedge worth testing.

HYPOTHESES, EACH WITH ITS TEST

HYPOTHESIS	TEST
Fewer claim-time support contacts and disputes	Four-week pilot at one claim-assistance desk, against a matched cohort
More eligible-room choices when the consequence is visible	Room-change rate in the simulator, then claims data
Panel insurers co-fund a coverage-clarity tool	Three conversations; one signed pilot letter is the pass
A coverage check lifts conversion or renewal on the policy page	A/B test, if Paytm chooses to run one

Revenue option · untested

Broker-side feature

Inside the health-policy journey and claim assistance; the return is retention and fewer disputes.

Revenue option · untested

Insurer-paid integration

Panel insurers co-fund coverage clarity for the policies they sell; a signed pilot letter is the pass.

Revenue option · untested

Premium claim-assistance tier

The question sheet and the evidence pack as part of a paid assistance service.

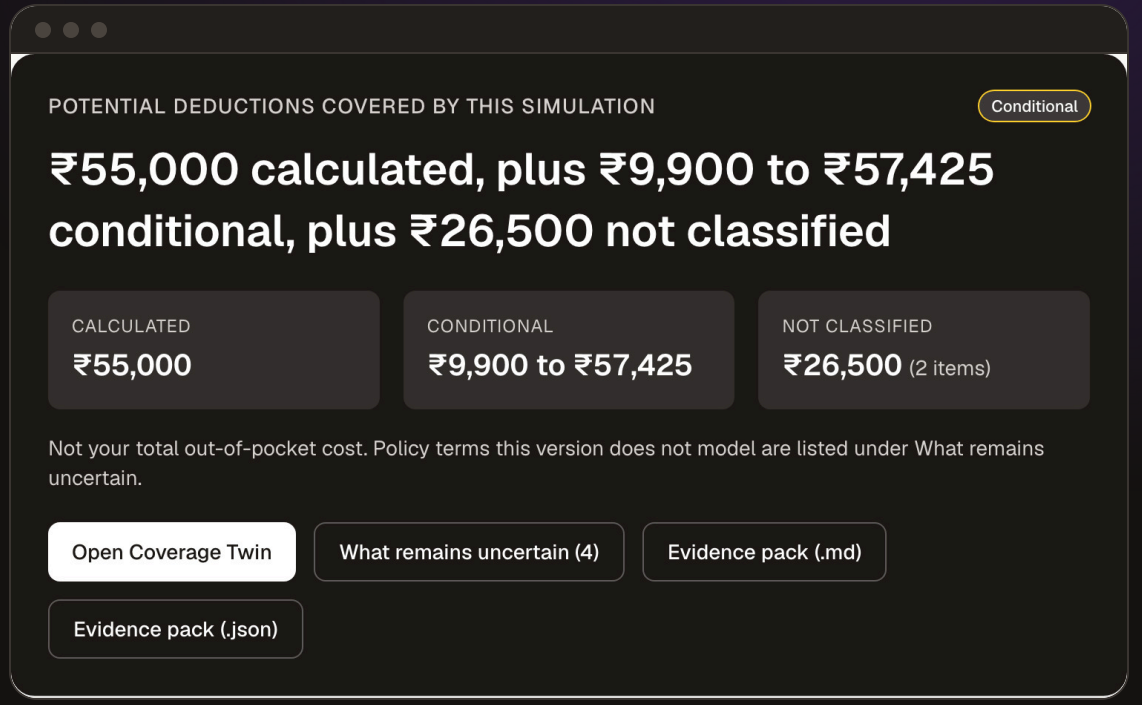
Which option, if any, depends on the tests above. No Paytm integration is claimed or built. Bima Sugam is not live; it belongs in the future tense.

Built, tested, runs with the Wi-Fi off

Shortlist us and we bring a laptop. Three minutes: change the room, open a number, watch the engine refuse to guess on a real wording.

- A second real wording through the builder, with the live adapter on.
- Independent review of the evaluation fixtures.
- A four-week pilot at a broker's claim-assistance desk.

Code, tests and documents: github.com/Pushkarmondal/paytm-hackathon



The same engine on the real wording: ₹55,000 calculated, a co-payment and two caveated items as ranges, two lines it will not classify.

Pushkar Mondal · Founding Engineer, Xeliport · October 2026